

Ureteral Reconstruction

WARWIKI

Patient Information · Surgery to repair a blocked or narrowed ureter

Ureteral reconstruction is surgery to repair a blocked or narrowed ureter, the tube that carries urine from the kidney to the bladder. The goal is to let urine drain freely and protect the kidney. How it is done depends on where the narrow spot is and how long it is.

About This Procedure

A **stricture** is a scarred, narrow spot. It traps urine behind it and can harm the kidney. The repair is matched to the stricture:

- **Short stricture:** the bad part is removed and the healthy ends are sewn together. Or the ureter is joined to the bladder in a new spot. Sometimes the bladder is stretched up to reach it.
- **Longer stricture:** the ureter is patched open with a **graft**, often from inside your cheek. This has its own handout.
- **Very long damage:** a piece of your own **bowel** replaces the missing part of the ureter.

A soft **stent** (a thin tube) holds the repair open while it heals. Most repairs are done with **robotic or keyhole surgery**. Some need a larger cut.

Benefits and Risks in Numbers

about
9 in 10

repairs work when the ureter's own ends or a cheek graft is used

about
8 in 10

bowel repairs for very long damage need no further open surgery

about
1 in 2

strictures stay open after treatment only through a scope (for comparison)

Numbers are averages from published studies. Your own risk may differ; ask your team.

Learn the Terms

Ureter

The tube that carries urine from the kidney to the bladder.

Stricture

A scarred, narrow spot that slows or blocks urine.

Stent

A soft tube inside the ureter, from kidney to bladder, that holds the repair open.

Graft

A small patch of your own tissue, often from inside the cheek, used to widen the ureter.

Bowel segment

A short piece of your own bowel used to bridge a long gap.

Reimplantation

Joining the ureter to the bladder in a new spot.

Hydronephrosis

Swelling of the kidney from urine backing up.

Cystoscopy

A look into the bladder with a thin scope. It is often how the stent is removed.

Will it hurt? You are asleep for the surgery, so you feel nothing. Afterward, the cuts will be sore. The stent can cause urgency or a twinge in your side when you urinate. If bowel is used, it takes a few days to start working again, and you may see mucus in your urine.

First days

Soreness and stent symptoms. Walk often and drink fluids.

Next few weeks

The stent stays in. Your team tells you when you can lift and exercise.

Stent removal

On the date your team gives you, often in the office.

After that

Scans check that the kidney drains well.

How to Get Ready

- Follow your instructions about eating, drinking and which medicines to stop, including blood thinners.
- A **urine test** checks for infection. Any infection is treated first.
- **Stop smoking.** It slows healing. Arrange a ride home.

Tell your team ahead of time if you:

- Take a blood thinner
- Have had belly or pelvic surgery or radiation

What Happens During Surgery

- 1 You are asleep and get antibiotics.
- 2 The surgeon finds the narrow part of the ureter.
- 3 The ureter is repaired over a stent: the ends are joined, patched with a graft, or bridged with bowel.
- 4 A small drain may be left for a short time.

About Your Stent

The stent stays in for several weeks. **Keep the removal date.** A stent left in too long can crust over and block the kidney. If bowel was used, you will also have blood tests over time.

Call the office that did your surgery if you have:

- A temperature of **100.4 °F (38 °C)** or higher, or chills
- Side or belly pain that is getting worse
- Redness, swelling or fluid leaking from a cut

Go to an emergency department if:

- You cannot urinate
- Urine like red wine or with clots
- Vomiting that will not stop, or no gas or stool (if bowel was used)
- You cannot reach the office and are worried

Three Things to Remember

1. The repair is chosen to fit your stricture: joining the ends, a cheek graft, or a piece of bowel.
2. Stent symptoms are normal. Keep the date to have the stent removed.
3. Go to your follow-up scans, even if you feel well.